Ysbyty Gwynedd
Emergency Laparotomy Pathway

This pathway is intended for all patients presenting with acute abdominal pain that may require emergency laparotomy

YG CG 415 Mar 2022 First Approved: Mar 2018

[1]SEPSIS 6
Within 1 hour:
Supplemental Oxygen
Lactate
Blood Cultures
IV Fluid Resuscitation
IV Antibiotics
Monitor Urine Output

Initial Assessment
- NEWS score within 30 minutes of admission
- FBC/U+E/LFT/CRP/Clotting/Amylase/G+S
- Lactate
- Sepsis screening & implementation of Sepsis 6[1]
- Rockwood Clinical Frailty Score (if ≥65 yrs)

[2]ESCALATION CRITERIA
Any of:
NEWS ≥5 or ≥3 in any 1 variable
Lactate >2
Pneumoperitoneum on CXR
Suspected Peritonitis
Refractory Abdominal Pain
Non-specific concern for patient

Escalation Criteria[2]

Absent

Surgical Review (ST3 or higher) within 2 hours of admission/referral
Review by/Discussion with Consultant Surgeon within 4 hrs

CT Scan within 12 hrs of initial surgical review

Present

Surgical Review (ST3 or higher) within ½ hour of admission/referral

Inform Consultant Surgeon immediately

Urgent CT scan:
Direct Senior Discussion with Radiologist

Patient too unwell to wait for CT scan

Detailed completion of CT request form is mandatory

CT Report within 1 hour
If Intra-abdominal sepsis: Verbal report to Consultant Surgeon

High Risk Indicators
- Escalation criteria present
- NELA Risk Score ≥5%
- ASA ≥3
- Insulin Dependant Diabetes
- eGFR ≤45 (CKD/AKI)
- Clinical Frailty Score ≥4
- Immunosuppressive therapy
- Perforation, Ischaemia

RISK ASSESSMENT
Document in Notes
Use Clinical Judgement
& High Risk Indicators

Mortality Risk: Low <5%

Decision made for Emergency Laparotomy
Upon Booking,: Use Urgency criteria: 1 < 2hrs, 2A 2-6hrs, 2B 6-18hrs, 3 >18hrs
Patients with Sepsis: <6hrs (Septic Shock <3hrs)

Mortality Risk: High ≥ 5%
Pre-Op review by Consultant Anaesthetist.
Consultant surgeon to directly contact Consultant Anaesthetist to discuss case.

Direct Consultant Anaesthetist & Surgeon Care
Goal Direct Fluid Therapy

Mortality Risk: High ≥ 25%
MDT Discussion with patient/family
Non-operative options & ceilings of care

Palliative Care: For advice
09:00-17:00: Bleep 183
Out of hours Advice: 01978 316800

POST OPERATIVE CARE

At Operation End:
Re-calculate Mortality Risk using the surgical findings

Ward

Critical Care

- Physiotherapy within 24 hours
- Refer for Care of The Elderly Team review if **CFS ≥5**
- Nutrition review
- Discharge planning